Metro Health Medical Center - Acute Care Pavilion

CLINICAL DOCUMENTATION — NARRATIVE INPATIENT DISCHARGE SUMMARY V1

Patient: Mary Johnson | **MRN:** PAT-60002 | **DOB:** 1972-11-12 | **Gender:** Female | **Doc ID:** NARRATIVE_0002
Encounter Date: 2025-02-09 | **Discharge/Note Date:** 2025-02-13 | **Provider:** Dr. Robert Evans, MD

CHIEF COMPLAINT

The patient presented with progressive shortness of breath and pleuritic chest discomfort.

HISTORY OF PRESENT ILLNESS

The patient is a 45-year-old female with a medical history pertinent for hypertension and dyslipidemia who presented for evaluation of progressive shortness of breath and pleuritic chest discomfort. Symptoms began approximately four days prior to admission and escalated in severity. On presentation to the emergency department, initial diagnostic evaluation demonstrated that glucose was 88.3 mg/dL. Concurrently, serum chemistry was notable for a potassium of 4.08 mmol/L. Vital signs upon arrival were stable, and the patient was admitted to the inpatient general medicine service.

HOSPITAL COURSE

During the hospital course, the inpatient team initiated targeted management. The patient received Omeprazole 20 mg oral once daily in the morning with good clinical tolerance. Serial assessments revealed gradual resolution of presenting symptoms. Prior to planned discharge, repeat laboratory testing confirmed that platelet count stabilized at 244.6 10*3/uL. The patient was ambulating without assistance and tolerating a regular diet.

DISCHARGE MEDICATIONS

The patient was discharged home in stable clinical condition with instructions to take Amoxicillin 875 mg oral every 8 hours for 10 days, and Cephalexin 250 mg PO QID for 7 days. Medication reconciliation was completed and all questions regarding dosing schedules were answered.

DISCHARGE INSTRUCTIONS & FOLLOW-UP

The patient was advised to follow up with their primary care physician within 7 to 10 days. Return precautions for chest pain, recurrent shortness of breath, or fever were provided and understood.